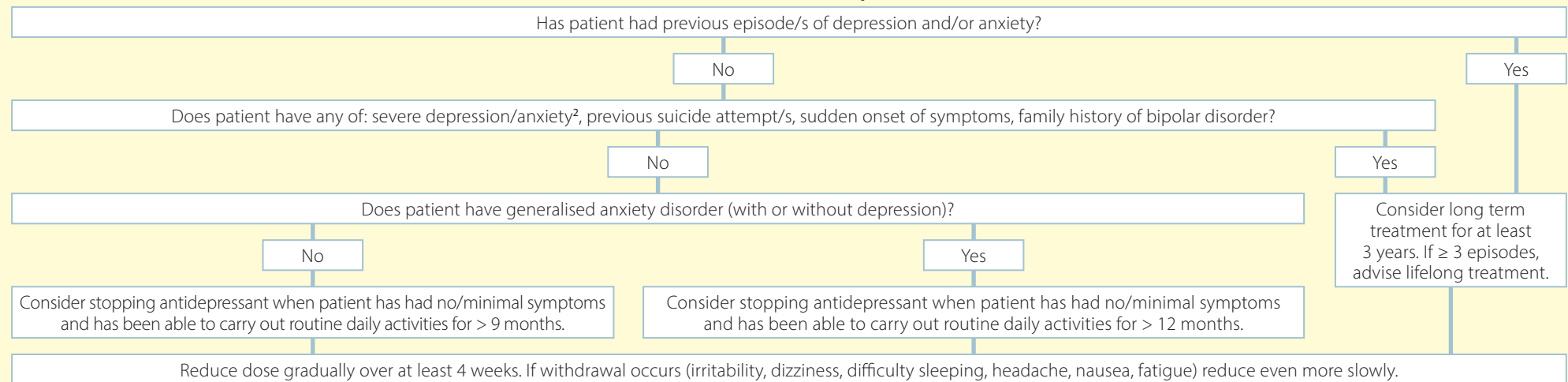


Treat the patient with depression and/or generalised anxiety

- Refer patient for counselling (ideally cognitive behavioural therapy or interpersonal therapy if available) and to social worker and/or helpline/support group ➤ 178.
- If occupational therapist (OT) available, refer for mood, self-esteem, motivation, coping skills and constructive use of leisure time.
- Discuss benefits of antidepressants for depression and generalised anxiety disorder. Respect the patient's decision if s/he declines antidepressants.
- If generalised anxiety disorder or severe anxiety¹ on starting antidepressant, consider **diazepam** 2.5-5mg daily as needed, for up to 10 days. Avoid if patient is known to use substances.
- Start **fluoxetine**. If fluoxetine poorly tolerated, give instead **citalopram**. If difficulty sleeping and sedating antidepressant desired *and* no suicidal thoughts, start instead **amitriptyline**.

Medication	Dose	Note	Side effects
Fluoxetine	Start 20mg on <i>alternate days</i> for 2 weeks, then increase to 20mg <i>daily</i> in the morning. If patient has increased anxiety, delay increase in dose for another 2 weeks.	• Explain that anxiety may increase initially and to return if severe. • Discuss with specialist if patient has epilepsy, liver or kidney disease. • Monitor glucose more often in diabetes. • Advise family to monitor and return if condition worsens (suicidal thoughts/ unusual changes in behaviour). • If patient unable to tolerate fluoxetine, stop fluoxetine and start citalopram 10mg next day.	Changes in appetite and weight, headache, restlessness, difficulty sleeping, nausea, diarrhoea, sexual problems.
Citalopram	Start 10mg daily for 1 week, then increase to 20mg daily.	Avoid if heart failure, arrhythmias, kidney failure.	Drowsiness, difficulty sleeping, headache, dry mouth, nausea, sweating, changes in appetite and weight.
Amitriptyline	Start 25mg at night. Increase by 25mg every 5 days. Review at 2 weeks: if good response, continue at this dose (75mg). If partial or no response, continue to increase by 25mg every 5 days as needed, up to 150mg/day.	Use if fluoxetine and citalopram contraindicated or poorly tolerated. Avoid if on bedaquiline, suicidal thoughts (can be fatal in overdose), heart disease, urinary retention, glaucoma, epilepsy and elderly patients.	Dry mouth, constipation, difficulty urinating, blurred vision, sedation

Decide duration of antidepressant



- Review 2 weekly, even if not on antidepressants, until symptoms get better, then monthly. Once stable, review 3-6 monthly.
- If no better after 8 weeks either on antidepressant or not, refer.

¹Patient has felt nervous, anxious or panicky or been unable to stop worrying or thinking too much. ²Patient has multiple depressive/anxiety symptoms, occurring nearly every day, that severely impairs daily functioning.